ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to [redacted] or FAX to [redacted] For referral questions, please contact an Illumination Referral Coordinator at [redacted]

Referring Party

Referring Hospit[redacted] Dept/Floo[redacted]
SW/CM/R[redacted] Phone/Pager [redacted]
[redacted] Nursing Station [redacted]
Authorized by:_____ Phone #:_____

Patient Information

Patient Nam[redacted] [redacted] Birth (dd/mm/yyy) [redacted]
Gender: ☑ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Afric[redacted]
Last 4 # of Social Securit[redacted] Insurance: CalOptima Preferred Language: English
Housing Situation: ☑ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Cellulitis
Admit [redacted] Any Wounds? ☑ Yes ☐ No
If yes, explain the number, location, size and stage:
Left ankle, ~~heel abrasion~~ cellulitis
Independent with Wound Care? ☑ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☑ No
Was PPD/TB Test or Chest X-Ray Performed? ☐ Yes ☑ No If yes, Date:_____ Results:_____

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	☐ Yes* ☑ No	Active Infection	☐ Yes* ☑ No
*Explain_____		*Explain_____	
Self Administer All Meds	☑ Yes ☐ No*	Ambulatory	☑ Yes ☐ No
Continent of Bowel & Bladder	☑ Yes ☐ No	Assistive Device	☐ Yes* ☑ No
Colostomy/Ileostomy	☐ Yes ☑ No	*Explain_____	
Foley Catheter	☐ Yes ☑ No	Hospice	☐ Yes* ☑ No
Diabetes	☐ Yes ☑ No	*Explain_____	
Insulin	☐ Yes ☑ No	Car	☐ Yes ☑ No
Diabetes related Oral Meds	☐ Yes ☑ No	Service Animal	☐ Yes ☑ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☑ No ☐ Yes

☑ Mental Health DX If checked, what type? bipolar
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain_____

Length of Stay Authorized in Recupertive Care: _____ Days

ILLUMINATION STAFF ONLY

Date Received:_____ Reviewed by:_____
Approved: ☐ Yes* ☐ No** *Date:_____ *Authorization #:_____ **Reason for Denial:_____
Admission Date:_____ Admission Time:_____ Exit Date:_____
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date:_____ by_____ Total LOS____

CalOptima Health

CalAIM Recuperative Care/Short-Term Post Hospitalization Referral/Authorization Request

Instructions:
Please complete this form and attach the following documents: Plan of Care, Discharge Plan, PT/OT notes and H&P. Authorization [redacted] be denied if this information is not provided.

Step 1: Complete all information below.

Referral Information

Referral [redacted]	Referred b [redacted]
Agency or relationship to membe [redacted]	
Referring provider National Provider Identifier (NPI) (if applicable):	
Phon [redacted]	[redacted]

Member Information

Member name [redacted]	CalOptima Health ID [redacted]
Date of birth [redacted]	Member's preferred language: English
Phone:-	Email:
Is the member currently in hospital? ☑ Yes ☐ No	
Last visit to PCP: Unknown	Next PCP visit: Unknown

Primary Care Provider (PCP) Information

Name: [redacted]	[redacted]
Phone:-	Email:

Step 2: [redacted] the boxes for the Community Supports the member is interested in receiving. The following pages provide additional eligibility information about Community Supports.

Step 3: Please check off the most appropriate Recuperative Care/Short-Term Post-Hospitalization Housing pathways based on the eligibility listed in the following pages.

Please note that members [redacted] not receive more than a combined 182 days of Short-Term Post-Hospitalization, Recuperative Care and Transitional Rent during any rolling 12-month period.

	Pathway	Description	Eligibility
☐	Recuperative Care only	Short-term residential care for individuals who no longer require hospitalization but will need to heal from an injury, illness or mental health condition.	Both must apply: Homeless or at risk of homelessness Member requires recovery in order to heal from an injury or illness.
☐	Short-Term Post-Hospitalization Housing only	Assists members with high medical or behavioral health needs with short-term housing after leaving the hospital, recovery facility, recuperative care or other facility.	Member must meet the following criteria: (1) Member is exiting an institution, which includes recuperative care facilities, inpatient hospitals, residential substance use disorder or mental health treatment facilities, correctional facilities or nursing facilities. **AND** (2) Member is experiencing homelessness. **AND** (3) Member meets one the following criteria: a. Is receiving ECM b. Have one or more serious chronic conditions c. Have a serious mental illness; or d. Is at risk of institutionalization or requiring residential services as a result of a substance use disorder. **AND** (4) Member is having ongoing physical and behavioral health needs as determined by a qualified health professional that would otherwise require continued institutional care if not for receipt of Short-Term Post-Hospitalization Housing.
☐	Nursing Facility with plans to transition to Recuperative Care	Short-term residential care for individuals who no longer require hospitalization but still need to heal from an injury or illness or a mental health condition.	Select one that applies: Homeless or at risk of homelessness Member who is at risk of hospitalization or at post-hospitalization Members who live alone with no formal supports.

☑	Recuperative Care with plans to transition to Short-Term Post-Hospitalization Housing	Short-term residential care for individuals who no longer require hospitalization but still need to heal from an injury, illness or mental health condition.	Select one that applies: Homeless or at risk of homelessness Individuals who are at risk of hospitalization or at post-hospitalization **OR** Individuals who live alone with no formal supports **AND** Member is exiting recuperative care, inpatient hospital, residential substance use disorder treatment facility, residential mental health treatment facility, correctional facility or nursing facility.

Admitting Diagnosis

Qualifying Recuperative Care/Short-Term Post-Hospitalization Housing diagnosis:

cellulitis

ED visit/hospital admit date: [redacted] Expected discharge date:

Will the member need any specialist follow-up care? ☐ Yes ☐ No

1) Specialty:	
Provider name:	
Phone:	
Scheduled appt date:	
2) Specialty:	
Provider name:	
Phone:	
Scheduled appt date:	
3) Specialty:	
Provider name:	
Phone:	
Scheduled appt date:	

Authorized Home Health Provider

Service(s): ☐ Physical therapy ☐ Occupational therapy ☐ Speech ☐ Wound care

☐ Personal care

Provider name: [redacted] [redacted] date:

Health Information

General

Gender: ☑ Male ☐ Female
☐ Placement needs related to gender.

COVID vaccine
Dose 1: ☐ Yes ☐ No
Dose 2: ☐ Yes ☐ No
Booster 1: ☐ Yes ☐ No
Booster 2: ☐ Yes ☐ No

TB test or chest X-ray performed? ☐ Yes ☑ No
If Yes, date:
Results: ☐ Positive
☐ Negative
Comments:

Neuro

Alert and oriented to: ☑ Person ☑ Place ☑ Time ☑ Situation

Respiratory

☐ Requires O2 (Explain):

GI/GU

☐ Incontinent of bowel
☐ Incontinent of bladder
☐ Colostomy/ileostomy
☐ Foley catheter

Does the member require tube feeding?
☐ Yes ☑ No

Ambulation/Mobility

Can the member independently perform ADLs? ☑ Yes ☐ No
How far can the member ambulate? 100+ feet

Does the member use DME? ☐ Yes ☑ No
If yes, please explain:

Fractures: ☐ Yes ☑ No

Recent surgery: ☐ Yes ☑ No

Integumentary

Wound(s): ☑ Yes ☐ No
If yes, provide location(s)/size/stage: left ankle cellulitis

Independent with wound care? ☑ Yes ☐ No

Infections

Communicable disease/isolation describe:

IV Antibiotics: ☑ Yes ☐ No
If yes, frequency: 8 hrs

Psycho-social Information:

☐ Registered sex offender

Member has:
☐ Car ☐ Spouse/partner
☐ Service animal ☐ Pets

Substance Use ☐ None

Type	Last Date Used
☐ Alcohol	
☐ Cocaine	
☐ Heroin	
☑ Methamphetamine	[redacted]
☐ Opioid	
☐ Other	

Mental Health DX

☐ Anxiety ☑ Bipolar ☐ Cognitive impairment ☐ Depression ☐ Schizophrenia
☐ Trauma-related ☐ Other (please explain):

☐ Current treatment:

Requires assistance with ADLs, please explain:

Medication Management

☐ Diabetic ☐ Insulin ☐ Oral meds ☐ Anticoagulants ☐ Requires INR/PTT checks
☐ Requires assistance with medication. List medication(s):

Does the member have enough medication to last through the end of the month? ☑ Yes ☐ No
Does the member understand how to obtain refills on their medications? ☑ Yes ☐ No

Health Information

Does the member have a preferred pharmacy? ☐Yes ☑No
If yes, where:
Does the member understand how to take their medication and why they are taking their medication? ☑Yes ☐No

Step 4: Based on the services selected for the member above, please submit this Referral Form to the most appropriate provider listed below via fax or mail.

Community Supports Provider Contact Information

Recuperative Care and Short-Term Hospitalization

[redacted]	[redacted]
Illumination Foundation (serving adults and children)	Phone number: [redacted] Fax number: [redacted] Email: [redacted]

Select from the below only after checking the capacity for OC Providers first.

[redacted]

STOP

Community Supports Provider to Complete Section Below

Step 5: Complete the section below and return the response to the referrer at the hospital or skilled nursing facility. If the member belongs [redacted] please submit these documents directly [redacted]

Accepting/Not Accepting

Was the member accepted? ☐Yes ☐No
If the member [redacted] service, please provide the reason:

[redacted]

Patient Name [redacted]
MRN [redacted]
Note Type: History and Physical: Internal Medicine

Admission Date/Time [redacted] **Age:** 36 years, **DOB** [redacted] **Be** [redacted] **Attending Physician** [redacted]

HISTORY AND PHYSICAL

DATE OF SERVICE: [redacted]

DATE OF ADMISSION: [redacted]

REASON FOR ADMISSION: L foot pain

HISTORY:
This 36-year-old male with a past medical history of bipolar disorder presents to the emergency room due to left foot pain after he stepped on something, he does not know when or what it was that he stepped on. Patient was very brief in his history, was unable to give me more information. Patient denies fevers chills. Tdap received in the emergency room.

PMH: As above
PSH: As above
Family History: Noncontributory
Social History: Denies substance use

14 point ROS negative except for above

CURRENT MEDICATIONS:

Active Medication List					
Ordered Medication	**Dosage**	**Route**	**Frequency**	**Start Date/ Time**	**Status**
VANCOMYCIN IVPB - 1000MG IN 250ML	167 ml/hr	IV PIGGY	X1	[redacted]	active
NS IV - 1000ML	100 ml/hr	INTRAVENOUS	CONTINUOUS		active
ACETAMINOPHEN(TYLENOL) TAB - 325MG	650 MG	ORAL	PRN Q6H		active
ONDANSETRON(ZOFRAN)INJ - 4MG/2ML	4 MG	IV PUSH	PRN Q4H		active
VANCOMYCIN IV PER PHARMACY PROTOCOL		IV PIGGY	RX PROTOCOL		active
PIP-TAZO(ZOSYN)IVPB-3.375GM/ NS 100ML 4H	25 ml/hr	IV PIGGY	Q8H		active
KETOROLAC(TORADOL)INJ - 15MG/1ML	15 MG	IV PUSH	PRN Q6H		active
MORPHINE INJ - 2MG/1ML	1 MG	IV PUSH	PRN Q4H		active

Electronically signed by [redacted]

[redacted]

Patient Name [redacted]
MRN [redacted]
Note Type: History and Physical: Internal Medicine

Active Medication List					
Ordered Medication	**Dosage**	**Route**	**Frequency**	**Start Date/ Time**	**Status**
VANCOMYCIN IVPB - 750MG IN 250ML	166 ml/hr	INTRAVENOUS	Q8H	[redacted]	active
NICOTINE(NICODERM CQ)PATCH - 21MG	21 MG	TOPICAL	DAILY		active

ALLERGIES:

Allergen: No Known Drug Allergies, **Type:** Medication, **Reaction:** , **Severity:**

REVIEW OF SYSTEMS:

Fourteen point review of systems negative except stated above

PHYSICAL EXAM:

VITALS:

Vital Signs									
Date/Time	**BP (mm/ Hg)**	**BP Position/ Site**	**MAP (mm/Hg)**	**Heart Rate**	**Resp**	**Temp (°F)**	**SPO2%**	**O2 Device**	**Blood Sugar**
[redacted]	98/60	LYING/R ARM	73	85	16	[redacted] ORAL	96 %	Room Air 21%	

Gen: Resting comfortably
HEENT: EOMI
Neck: Supple
CV: RRR, no m/r/g
Resp: CTA B/l, no w/r/r. respirations nonlabored
Abd: Soft, nontender, nondistended
Ext: L foot with medial side punctate ulcerations and mild erythema
Neuro: Alert and Oriented. Moves all extremities spontaneously.

LABORATORY:

Lab Results (Last 24hrs)					
Test	**Results**	**Units**	**Reference Range**	**Collected**	**Status**
WBC	18.5 H	K/CUMM	[redacted]	[redacted]	final
RBC	[redacted] L	M/CUMM	[redacted]		final
HGB	[redacted] L	g/dL	[redacted]		final
HCT	[redacted] L	%	[redacted]		final
MCV	[redacted]	fL	[redacted] H=100		final

Electronically signed by [redacted]

[redacted]

Patient Name [redacted] **DOB** [redacted]
MRN [redacted]
Note Type: History and Physical: Internal Medicine

Lab Results (Last 24hrs)					
Test	**Results**	**Units**	**Reference Range**	**Collected**	**Status**
MCH	[redacted]	pg	[redacted]	[redacted]	final
MCHC	[redacted]	g/dL	[redacted]		final
RDW	**15.4 H**	%	[redacted]		final
PLT	319	K/CUMM	L=150 H=450		final
MPV	[redacted]	fL			final
MANUAL DIFF	AUTOMATED MET				final
NEUTROPHIL	**88 H**	%	L=47 H=82		final
LYMPHOCYTE	**5 L**	%	L=15 H=43		final
MONOCYTE	5	%	L=3 H=10		final
EOSINOPHIL	1	%	L=0 H=4		final
BASOPHIL	0	%	L=0 H=1		final
BAND			L=0 H=4		final
METAMYELOCYTE					final
MYELOCYTE					final
PROMYELOCYTE					final
BLASTS					final
ATYP LYMPH					final
OTHER CELLS					final
NRBC					final
ABSOLUTE NEUTROPHIL	[redacted] **H**	K/CUMM	[redacted]		final
ABSOLUTE LYMPHOCYTE	[redacted]	K/CUMM	[redacted]		final
ABSOLUTE MONOCYTE	[redacted]	K/CUMM	[redacted]		final
ABSOLUTE EOSINOPHIL	[redacted]	K/CUMM	[redacted]		final
ABSOLUTE BASOPHIL	[redacted]	K/CUMM	[redacted]		final
PLT EST					final
PLT MORPH					final
RBC MORPH					final
FASTING	UNK				final
SODIUM	138	mmol/L	L=137 H=145		final
POTASSIUM	[redacted]	mmol/L	[redacted]		final
CHLORIDE	100	mmol/L	L=98 H=107		final
CO2	24	mmol/L	L=22 H=30		final
GLU	**148 H**	mg/dL	L=70 H=99		final
BUN	**7 L**	mg/dL	L=9 H=20		final
CREATININE	[redacted] **L**	mg/dL	[redacted]		final
AGE	36	yrs			final

Electronically signed by [redacted]

[redacted]

Patient Name [redacted] **Account #** [redacted]
MRN [redacted]
Note Type: History and Physical: Internal Medicine

Lab Results (Last 24hrs)					
Test	**Results**	**Units**	**Reference Range**	**Collected**	**Status**
gfr	162			[redacted]	final
GFR	>60				final
ALBUMIN	[redacted]	g/dL	[redacted]		final
CALCIUM	[redacted]	mg/dL	[redacted]		final
SGOT/AST	27	IU/L	L=17 H=59		final
ALKALINE PHOS	84	U/L	L=36 H=126		final
TOTAL BILI	**1.7 H**	mg/dL	[redacted]		final
TOTAL PROTEIN	**8.5 H**	g/dL	[redacted]		final
SGPT/ALT	22	U/L	L=0 H=50		final
LACTIC ACID	[redacted] **HH**	mmol/L	[redacted]		final
CALLED TO:	[redacted]				final
TIME(HHmm):	0014				final
DATE(mmddYY):	[redacted]				final
INITIALS:					final
REP/VERIFIED	YES				final
READ BACK	YES				final
BLOOD CULTURE AEROBIC ANAEROBIC					registered
BLOOD CULTURE AEROBIC ANAEROBIC					registered
LACTIC ACID REFLEX	[redacted]	mmol/L	[redacted]		final

RADIOGRAPHIC DATA:

MRI LLE

IMPRESSION: 1. Soft tissue swelling and edema. 2. No evidence of osteomyelitis on this examination.

ASSESSMENT AND PLAN:

#Sepsis
#L foot Cellulitis
#Leukocytosis
- TDAP given in ED
- Vanc/zosyn
- MRI LLE -> no osteo/abscess
[redacted]iatry consult
- blood cultures

Electronically signed by [redacted]

#bipolar
- Restart home meds

F: SLIV
E: Replete PRN
N: Renal
GI: Protonix
PPX: Heparin

Portions of this chart [redacted] have been created with voice recognition software. Occasional wrong-word or -sound-alike- substitutions [redacted] have occurred due to the inherent limitations of voice recognition software. Please read the chart carefully and recognize, using context, where these substitutions have occurred.

[redacted]

PATIENT NO [redacted]	ADMIT DATE/TIME [redacted]	MED REC NO [redacted]
ROOM/BED [redacted]	DISCH DATE/TIME:	PREADMIT DATE [redacted]
STAY TYPE: 1 I/P	SERVICE CODE: 12 IP M/S	ADMIT SOURCE: ER
		BY: [redacted] PREV/ADM: [redacted]

PATIENT NAME [redacted]		CONFIDENTIAL PT: [redacted]
PT ADDRESS 1 [redacted]	BIRTH SEX: M	DOB [redacted]
PT ADDRESS 2	GENDER IDENTITY: Not Answered	
CITY/ST/ZIP		LANGUAGE: English
BIRTHPLACE	RLG: NO PREFERENCE	AGE: 36
PHONE	SS#: [redacted]	RACE: 2
CELL PHONE		ADV/DIR: N
SPOUSE NAME:		[redacted]
EMAIL: NO EMAIL		ABM:

EMER CONTACT [redacted]	RELATIONSHIP: ECON
ADDRESS 1	HOME PH [redacted]
ADDRESS 2	WORK PH
CITY/ST/ZIP	CELL PH

GUARNATOR NAME [redacted]	[redacted] PATIENT:
ADDRESS 1	SS# [redacted]
ADDRESS 2	PHONE
CITY, ST, ZIP	OCCUPATION:
EMPLOYER: UNEMPLOYED	EMPLOYER PHONE:

ATTENDING PHYSICIAN [redacted]	PHONE [redacted]
ADMITTING/REFERRAL PHY: ZZI NO IPA	PHONE:
PRIMARY CARE PHYSICIAN: ZZ NON STAFF	PHONE:

PRIMARY INSURANCE: CAL OPTIMA DIRECT -I/P	POLICY NUMBER [redacted]
BILL ADDRESS 1: [redacted]	PRECERT #
BILL ADDRESS 2:	FINANCIAL CLASS: LK
CITY, ST, ZIP:	BILL PHONE:
GROUP NAME: [redacted]	GROUP NO: NA
INSURED: [redacted]	SEX/REL: M 18
EMPLOYER: UNEMPLOYED	DOB [redacted]
ADDRESS:	EMPLOYER PHONE:
CITY/ST/ZIP:	EID/ESC: / 3
IPA/BENEFIT PLAN:	

SECONDARY INSURANCE:	POLICY NUMBER:
BILL ADDRESS 1:	PRECERT #:
BILL ADDRESS 2:	FINANCIAL CLASS:
CITY, ST, ZIP:	BILL PHONE:
GROUP NAME:	GROUP NO:
INSURED:	SEX/REL:
EMPLOYER:	DOB:
ADDRESS:	EMPLOYER PHONE:
CITY/ST/ZIP:	EID/ESC: /
IPA/BENEFIT PLAN:	

CHIEF COMPLAINT: CELLULITIS

ADMIT DIAGNOSIS: [redacted] - Cellulitis of left lower limb

MRSA	VRE	COVID VACCINE DATE	PNEUMONIA VACCINE - DATE
		[redacted]	

[redacted]

[redacted]

[redacted]

AGE: 36 SEX: M

[redacted]

ALLERGIES: No Known Dru

PATIENT PROGRESS NOTES

[redacted]

[redacted]

Physical Therapy

[redacted]

[redacted]

Physical Therapy

Precautions: Fall risk.
Rolling: Independent.
Supine to Sit: Independent.
Sit to Supine: Independent.
Sitting Balance: Good.
Sitting Tolerance: minutes.
Sitting Position: edge of bed.
Sit to Stand/Stand to Sit: Independent.
Standing Balance: Good with assistive device.
Transfers to/From Bed: Independent.
Ambulation {assist level}
Supervised ambulation is required due to the patient's legal blindness; walker management is necessary.
Distance: 800 feet.
Equipment For Ambulation: front wheeled walker.
Therapeutic exercise performed LE's.
Monitored values No pain.
Patient/Family instruction given
Educated on PT plan/purpose, Pt stated/demo'd good understanding.
Therapist notes:
Pt scheduled for I&D this afternoon. Dr requesting documentation for recuperative care. Pt is {I} with TE, bed mob and transfers {he states he quickly memorizes his surroundings and gains independence quickly.} He is having no pain today and requested to keep walking during gait session. After ambulating 400ft he asked to keep going. He meets requirements for recuperative care. Pt lost his white cane and inquired if we could help him get another.
DME Recommendation FWW front wheeled walker.
DC Recommendation Shelter/Recuperative care.
Functional training {mins} 15 mins.
Gait training {mins} 15 mins.
Therapeutic exercise {mins} 15 mins.
Total Session Duration Summation: [redacted]
Room number/Room change [redacted]

[redacted]

AGE: 36 SEX: M

[redacted]

ALLERGIES: No Known Dru

PATIENT PROGRESS NOTES

[redacted]

[redacted]

Wound Care

[redacted]

[redacted]

Wound Care

Wound Location: As itemized:

1. L Heel
2. L Med Ankle
3. L Med Foot

Wound Type: Cellulitis.

Wound Care Nurse's Notes/Comments:

Wound care consultation received: Braden score = 21

Communicated [redacted] primary nurse and confirmed PENDING podiatry consultation wi[redacted]r tx and evaluation.

[redacted]

ChartLink Physician Entered Orders
Date Range: [redacted]
Run Date/Time: [redacted]

Account: [redacted]
MR#: [redacted]
Allergies: No Known Drug Allergies

Age: 36
Sex: M
Room: [redacted]

ORDER Dt/Tm	START Dt/Tm	
[redacted]		Nursing Order
		Admit patient as: IP MSU
		Phone order entered by [redacted] Readback successfully Signed: Pending
[redacted]		Nursing Order
		Routine vital signs, per unit policy
		Phone order entered by [redacted] Readback successfully Signed: Pending
[redacted]		Nursing Order
		VTE: Low VTE risk, early ambulation
		Phone order entered by [redacted] Readback successfully Signed: Pending
[redacted]		Nursing Order
		Consult Podiatry, Dr: [redacted]
		Phone order entered by [redacted] Readback successfully Signed: Pending
[redacted]		Nursing Order
		Draw Vancomycin trough level [redacted] min prior to 0900 dose on [redacted]
		Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		C WITH DIFF	1		1		ROUTINE

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		C WITH DIFF	1		1		ROUTINE

Signed: [redacted]

[redacted] Age: 36

ChartLink Physician Entered Orders
Date Range: [redacted]
Run Date/Time: [redacted]

Account: [redacted]
MR#: [redacted]
Allergies: No Known Drug Allergies

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		C WITH DIFF	1		1		ROUTINE

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		SA SCREEN (NARES)	1	1X	1		ROUTINE

SPECIMEN TYPE: _nares_____
DETAILED DESCRIPTION: _nares_____

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		MPREHENSIVE METABOLIC PANEL	1		1		ROUTINE

{FASTING _____

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		SIC METABOLIC PANEL	1		1		ROUTINE

{FASTING _____

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		SIC METABOLIC PANEL	1		1		ROUTINE

{FASTING _____

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		VANCOMYCIN TROUGH	1	1X	1		ROUTINE

{ADMIN TIME _0900___
{DOSE AMT _750MG____

[redacted]

[redacted] Age: 36

ChartLink Physician Entered Orders Account: [redacted]
Date Range: [redacted] MR#: [redacted]
Run Date/Time: [redacted] Allergies: No Known Drug Allergies

Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		LOW EXT(JNT)W&W/O CONT LT	1	1X			

MR LOW EXT(JNT)W&W/O CONT LT
{REASON FOR PROCESS: SWELLING
Transportation: _WHEELCHAIR_____ IV? _Y_ 02? _N_ MRSA/VRE? _N_
COMMENT: _____

[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		I ANKLE W/O CONTRAST LT	1	1X			

MRI LEFT ANKLE W/O CONTRAST
{REASON FOR PROCESS: PAIN
Transportation: _WC_____ IV? _Y_ 02? _N_ MRSA/VRE? _N_
COMMENT: _NEED_LT_HEEL_AND_LT_ANKLE_____

[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		I ANKLE W/O CONTRAST LT	1	1X			STAT

MRI LEFT ANKLE W/O CONTRAST
{REASON FOR PROCESS: PAIN
Transportation: _WC_____ IV? _Y_ 02? _N_ MRSA/VRE? _N_
COMMENT: _NEED_LT_HEEL_AND_LT_ANKLE_____

[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		ECIAL INSTRUCTIONS	1	1X			

{SPECIAL INSTRUCTIONS _DOUBLE_PORTION_MEALS_____
OTHER INSTRUCTIONS: _____

[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

[redacted] Age: 36

ChartLink Physician Entered Orders Account: [redacted]
Date Range: [redacted] MR#: [redacted]
Run Date/Time: [redacted] Allergies: No Known Drug Allergies

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		ET: REGULAR	1	1X			

{ISOLATION: _____ 24HR FLUID RESTRICTION: _____ ML
{LIQUID CONSISTENCY: _____
{DIET CONSISTENCY: _REGULAR_____
{Start Meal: _BREAKFAST____ OTHER INSTRUCTIONS: _____

[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		TRITION CONSULT (MD ORDER)	1	1X			

{REASON FOR CONSULT: _MALNUTRITION_____
BMI<19

[redacted]

Protocol order entered b[redacted] Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		CIAL SERVICES CONSULT	1	1X			

{Reason SS Consult: _Homelessness_____
Comments: _smoking_cessation_____

[redacted]

Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		UND CARE CONSULT	1	1X			

{WOUND CARE: _CONSULT_____
{REASONS: _PER_PROTOCOL_____
{SITE: _LEFT_ANKLE_____ WOUND TYPE: _CELLULITIS_____
DID PATIENT COME FROM SNF? _N_ IF YES, NAME OF SNF:_____

[redacted]

Protocol order entered b[redacted] Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Desc/Questions	Qty	Freq	Duration	Days	Priority
[redacted]		CIAL SERVICES-HOMELESS	1	1X			

[redacted]

Protocol order entered b[redacted] Signed: Pending

[redacted]

ChartLink Physician Entered Orders
Date Range: [redacted]
Run Date/Time: [redacted]

Account: [redacted]
MR#: [redacted]
Allergies: No Known Drug Allergies

Age: 36
Sex: M
Room: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Rate	Route	Duration	Priority
[redacted]		NS IV - 1000ML		100 ML/HR	IV		ROUTINE
			CONTINUOUS				
		Add: NS IV - 1000ML	1000 ML				

Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		ACETAMINOPHEN(TYLENOL) TAB - 325MG	650 MG	PO		ROUTINE
			PRN Q6H			

FOR >100.8/Mild pain DOSE = 650MG = 2TABS
NTE 4 GM OF APAP IN 24 H RS
Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		ONDANSETRON(ZOFRAN)INJ - 4MG/2ML	4 MG	IVP		ROUTINE
			PRN Q4H			

FOR NAUSEA/VOMITING IV PUSH OVER [redacted] MINUTES

Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Rate	Route	Duration	Priority
[redacted]		VANCOMYCIN IV PER PHARMACY PRO		AS DIRECTED	IVPB		ROUTINE
			RX PROTOCOL				
		Add: VANCOMYCIN IV PER PHARMACY PROTOCOL	1 UNIT				

PHARMACY USE ONLY;

Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Rate	Route	Duration	Priority
[redacted]		PIP-TAZO(ZOSYN)IVPB-3.375GM/NS 1		0025 ML/HR	IVPB		ROUTINE
		Add: PIPERACILLIN-TAZO(ZOSYN)INJ - 3.375GM VL	3.375 GM				
		Add: NS IVPB - 100ML	100 ML				

INFUSE OVER 4 HOURS
Phone order entered by [redacted] eadback successfully Signed: Pending

(Order Changed)----------(Order Changed)

[redacted]		PIP-TAZO(ZOSYN)IVPB-3.375GM/NS 1		25 ML/HR			
			Q8H				

ChartLink Physician Entered Orders
Date Range: [redacted]
Run Date/Time: [redacted]

[redacted]
Account: [redacted]
MR#: [redacted]
Allergies: No Known Drug Allergies

Age: 36
Sex: M
Room: [redacted]

Add: PIPERACILLIN-TAZO(ZOSYN)INJ - 3.375GM VL 3.375 GM
Add: NS IVPB - 100ML 100 ML
Changed by: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		KETOROLAC(TORADOL)INJ - 15MG/1ML	15 MG PRN Q6H	IVP		ROUTINE

FOR Moderate___ PAIN [redacted]

Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		MORPHINE INJ - 2MG/1ML	1 MG PRN Q4H	IVP		ROUTINE

FOR Severe PAIN *FALL PRECAUTIONS* [redacted]
BBW
Phone order entered by [redacted] eadback successfully Signed: Pending

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Rate	Route	Duration	Priority
[redacted]		VANCOMYCIN IVPB - 750MG IN 250	Q8H	166 ML/HR	IV		ROUTINE

Add: VANCOMYCIN INJ - 750MG VIAL 750MG
Add: NS IVPB - 250ML 250ML
(PP)

[redacted]
Signed: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		NICOTINE(NICODERM CQ)PATCH - 21MG	21 MG DAILY	TOPICAL		ROUTINE

REMOVE AFTER 24 HOURS INSTRUCT PATIENT NOT TO SMOKE WHILE WEARING PATCH [redacted]
Phone order entered by [redacted] Readback successfully Signed: Pending

(Order Changed)----------(Order Changed)

[redacted] NICOTINE(NICODERM CQ)PATCH - 21MG 21 MG DAILY TOPICAL
Changed by: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		SERTRALINE(ZOLOFT)TAB - 50MG	100 MG DAILY	PO		ROUTINE

BBW [redacted]

[redacted]

Age: 36

ChartLink Physician Entered Orders
Date Range: [redacted]
Run Date/Time: [redacted]

Account: [redacted]
MR#: [redacted]
Allergies: No Known Drug Allergies

Signed: [redacted]

(Order Changed)----------(Order Changed)

[redacted] SERTRALINE(ZOLOFT)TAB - 50MG 100 MG DAILY PO

Changed by: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Route	Duration	Priority
[redacted]		OLANZAPINE (ZYPREXA) TAB - 5MG	5 MG DAILY	PO		ROUTINE

BBW

Signed: [redacted]

(Order Changed)----------(Order Changed)

[redacted] OLANZAPINE (ZYPREXA) TAB - 5MG 5 MG DAILY PO

Changed by: [redacted]

ORDER Dt/Tm	SCH Dt/Tm	Medication	Dose/ Freq	Rate	Route	Duration	Priority
[redacted]		VANCOMYCIN IVPB - 1000MG IN 250	Q8H	167 ML/HR	IVPB		ROUTINE

Add: VANCOMYCIN INJ - 1 GM VIAL 1000MG
Add: NS IVPB - 250ML 250ML
(PP)

Signed: [redacted]

Medication	Dose/ Frequency	Rate	Route	Action	New Stop Dt/Tm
VANCOMYCIN IVPB - 750MG IN 250ML	750	166 ML/HR		[redacted]	

Signed: [redacted]

Procedure	Action	New Stop Dt/Tm
MRI ANKLE W/O CONTRAST LT 1X	Discontinued [redacted]	[redacted]

Phone order entered by [redacted] Readback successfully Signed: Pending

[redacted]

----PATIENT NAME---- SEX AGE BIRTH ADMIT M/E# PATIENT# PM/LOC TYPE

[redacted]

[redacted] SEC: ZZI NO IPA PRI: ZZ NON STAF

PAT PHONE [redacted]

==========

---PROCEDURE--- MRSA SCREEN (NARES) [redacted]

--ORDERED-- --COLLECTED-- --REC'D-- --RESULTED-- --VERIFIED---

[redacted]

==========

MRSA SCREEN (NARES)

{ CULTURE SOURCE: _NARES_

{ MRSA SCREEN: _NEGATIVE_FOR_MRSA_COLONIZATION_ [redacted]

{ COMMENTS : _____

{ REPORTED TO IC ____ [redacted]

[redacted]

----PATIENT NAME---- SEX AGE BIRTH ADMIT M/R# PATIENT# [redacted]

[redacted] SEC: ZZI NO IPA PRI: ZZ NON STAF

PAT PHONE [redacted]

==========

---PROCEDURE--- BLOOD CULTURE AEROBIC ANAEROBIC [redacted]

--ORDERED-- --COLLECTED-- --REC'D-- --RESULTED-- --VERIFIED---

[redacted]

==========

BLOOD CULTURE AEROBIC & ANAEROBIC

{ CULTURE SOURCE: _BLOOD_

{ FINAL RESULTS: _PENDING__ (NORMAL: NEGATIVE)

{ PRELIMINARY RESULTS: _NEGATIVE_ (NORMAL: NEGATIVE) [redacted]

PRELIMINARY REPORT
__NO GROWTH DAY 3 (72 hrs)_
DATE: _____
TIME: _____
PRELIMINARY REPORT
_____NO GROWTH DAY 2_____
DATE: _____
TIME: _____
PRELIMINARY REPORT
_____NO GROWTH DAY 1_____
DATE: _____
TIME: _____

{ GRAM STAIN:

_____
_____
_____
_____
_____

{ CULTURE RESULTS:_____

_____
_____
_____
_____
_____
_____

NAME. [redacted] SEX.....: M ATTENDING: [redacted]
ACCT# [redacted] AGE.....: 36 Y [redacted] IPA
ROOM. [redacted] - PENDING ORDERS DOB..... [redacted] PRIM CARE.:ZZ NON STAFF
PAT. PHONE
ADMIT: [redacted]

HEMATOLOGY

[redacted]

	0820	2258	REFERENCE RANGE	UNITS
WBC	**13.1 H**	[redacted] **H**	[redacted]	K/CUMM
RBC	[redacted] **L**	[redacted] **L**	[redacted] - [redacted]	M/CUMM
HGB	[redacted] **L**	[redacted] **L**	[redacted]	g/dL
HCT	[redacted] **L**	[redacted] **L**	[redacted]	
MCV	[redacted]	[redacted]	[redacted] - 100	fL
MCH	[redacted]	[redacted]	[redacted] - [redacted]	pg
MCHC	[redacted]	[redacted]	[redacted] - [redacted]	g/dL
RDW	[redacted] **H**	[redacted]	[redacted]	
PLT	247	319	150 - 450	K/CUMM
MPV	[redacted]	[redacted]		fL
MANUAL DIFF	AUTOMATED MET	AUTOMATED MET		
NEUTROPHIL	72	**88 H**	47 - 82	%
LYMPHOCYTE	16	**5 L**	15 - 43	%
MONOCYTE	8	5	3 - 10	%
EOSINOPHIL	3	1	0 - 4	%
BASOPHIL	0	0	0 - 1	%
ABSOLUTE NEUT	[redacted] **H**	[redacted] **H**	[redacted] - [redacted]	K/CUMM
ABSOLUTE LYMP	[redacted]	[redacted]	[redacted] - [redacted]	K/CUMM
ABSOLUTE MONO	[redacted]	[redacted]	[redacted] - [redacted]	K/CUMM
ABSOLUTE EOSI	[redacted]	[redacted]	[redacted] - [redacted]	K/CUMM
ABSOLUTE BASO	[redacted]	[redacted]	[redacted] - [redacted]	K/CUMM

CHEMISTRY

[redacted]

	0820	0245	2258	REFERENCE RANGE	UNITS
SODIUM	139		138	137 - 145	mmol/L
POTASSIUM	[redacted]		[redacted]	[redacted] - [redacted]	mmol/L
CHLORIDE	**109 H**		100	98 - 107	mmol/L
CO2	22		24	22 - 30	mmol/L
GLU	86		**148 H**	70 - 99	mg/dL
BUN	**5 L**		**7 L**	9 - 20	mg/dL
CREATININE	**0.6 L**		**0.6 L**	[redacted] - [redacted]	mg/dL
GFR	>60		>60		
TOTAL PROTEIN	[redacted]		[redacted] **H**	[redacted] - [redacted]	g/dL
ALBUMIN	**3.1 L**		[redacted]	[redacted] - [redacted]	g/dL
CALCIUM	**8.3 L**		[redacted]	[redacted]	mg/dL
CORRECTED CAL	[redacted]			[redacted]	mg/dL
TOTAL BILI	[redacted]		[redacted]	[redacted] - [redacted]	mg/dL
ALKALINE PHOS	70		84	36 - 126	U/L
SGOT/AST	37		27	17 - 59	IU/L
SGPT/ALT	39		22	0 - 50	U/L
LACTIC ACID			[redacted] **HC**	[redacted] - [redacted]	mmol/L

LEGEND: L-Low, H-High, C-Critical, A-Abnormal, *E-Error

NAME. [redacted]
ACCT# [redacted]
ROOM. [redacted] [redacted] - PENDING ORDERS
ADMIT: [redacted]

SEX.....: M
AGE.....: 36 Y
DOB..... [redacted]
PAT. PHONE
MR#..... [redacted]

ATTENDING: [redacted]
SECOND...: ZZI NO IPA
PRIM CARE.:ZZ NON STAFF

CHEMISTRY

	[redacted] 0820	0245	2258	REFERENCE RANGE	UNITS
LACTIC ACID R		[redacted]		[redacted] [redacted]	mmol/L
CALLED TO:			[redacted]		
DATE(mmddYY):			[redacted]		
INITIALS:			[redacted]		
READ BACK			YES		
REP/VERIFIED			YES		
TIME(HHmm):			0014		

THESE ARE NEW REFERENCE RANGES PER THE AMERICAN DIABETES ASSOCIATION GUIDELINES.

FASTING GLUCOSE RANGES	INDICATIONS
[redacted] md/dL	NORMAL GLUCOSE, NOT DIABETIC
100 - 125 mg/dL	IMPAIRED FASTING GLUCOSE, OR PREDIABETIC
126 mg/dL or higher	DIABETES

Serum Creatinine is now standardized to the internaltionally accepted IDMS reference method. This is expected to improve the detection, diagnosis and treatment of chronic kidney disease by reducing inter-laboratory bias. Please note the new normal ranges.

GFR REFERENCE RANGE: > OR = 60
NOTE: IF THE PATIENT IS AFRICAN AMERICAN, PLEASE MULTIPLY THIS RESULT BY [redacted]
THIS RESULT HAS BEEN CALCULATED ASSUMING THE PATIENT IS NON-AFRICAN AMERICAN.
GFR is estimated by the MDRD Study equation using serum creatinine, age and gender. It is not adjusted for body size. The Cockcroft-Gault equation is recommended when calculating GFR for medication dosing in adults and for patients at risk of radiocontrast induced nephropathy.

The reported serum calcium has not been corrected. The following formula is recommended to correct serum calcium in patients with abnormal albumin levels:
Corrected calcium = [redacted] (4 - Patient's Albumin) + measured calcium where 4 equals the average albumin level in g/dL

NAME. [redacted] SEX.....: M ATTENDING: [redacted]
ACCT# [redacted] AGE.....: 36 Y [redacted] IPA
ROOM. [redacted] - PENDING ORDERS DOB..... [redacted] PRIM CARE.:ZZ NON STAFF
PAT. PHONE
ADMIT: [redacted] MR#..... [redacted]

TDM & TOXICOLOGY

[redacted]

	0820	REFERENCE RANGE	UNITS
VANCOMYCIN TR	[redacted]	[redacted] - [redacted]	ug/mL

MICROBIOLOGY

--ORDERED--	--COLLECTED--	--REC'D--	--RESULTED--	--VERIFIED---
[redacted]				0000

...

BLOOD CULTURE AEROBIC & ANAEROBIC

CULTURE SOURCE: BLOOD

FINAL RESULTS: PENDING (NORMAL: NEGATIVE)

PRELIMINARY RESULTS: NEGATIVE (NORMAL: NEGATIVE)
PRELIMINARY REPORT
NO GROWTH DAY 1
DATE:
TIME:

GRAM STAIN:

CULTURE RESULTS:

NAME. [redacted] SEX.....: M ATTENDING: [redacted]
ACCT# [redacted] AGE.....: 36 Y SECOND...: ZZI NO IPA
ROOM. [redacted] [redacted] - PENDING ORDERS DOB..... [redacted] PRIM CARE.:ZZ NON STAFF
PAT. PHONE
ADMIT: [redacted] MR#..... [redacted]

MICROBIOLOGY

--ORDERED--	--COLLECTED--	--REC'D--	--RESULTED--	--VERIFIED---
[redacted]				0000
[redacted]				...

BLOOD CULTURE AEROBIC & ANAEROBIC

CULTURE SOURCE: BLOOD

FINAL RESULTS: PENDING (NORMAL: NEGATIVE)

PRELIMINARY RESULTS: NEGATIVE (NORMAL: NEGATIVE)
PRELIMINARY REPORT
NO GROWTH DAY 1
DATE:
TIME:

GRAM STAIN:

CULTURE RESULTS:

[redacted]

GRAPHIC AND I & O

[redacted] Page 1 of 1

(TEMPORARY)

[redacted]

AGE: 36 SEX: M

ALLERGIES: No Known Dru

[redacted]

Date	[redacted]						[redacted]											
	A.M.			P.M.			A.M.			P.M.			A.M.			P.M.		
Hour	0-4	[redacted]	[redacted]	[redacted]	[redacted]	20-0	0-4	[redacted]	[redacted]	[redacted]	[redacted]	20-0	0-4	4-8	8-12	12-16	16-20	20-0
TEMPERATURE C / F																		
Pulse Rate		* 85		86	85	78		72										
Respiration		* 18		18	16	16		18										
B.P. - Sys/Dias		* 95/46		101/58	98/60	98/54		105/65										
O2LM																		
FiO2																		
O2SAT		* 96		98	96	97		100										
O2 Method		RA		RA	RA	RA		RA										
EtCO2																		
Weight / BSA / BMI	[redacted] 127 LBS 9.12 OZ KGS:62.4 GM					BSA:1.78	[redacted] 142 LBS 10.2 OZ GM Bed Scale					BSA [redacted] BMI:						
Patient Diet & Percent Consumed	08:45 DIET: REGULAR 100 % 13:00 DIET: REGULAR 100 % 19:25 DIET: REGULAR 50 %																	

BIP-Bi-Pap CP-C-Pap HF-High Flow Nasal Cannula NRM-Non-Rebreather Mask NC-O2 Cannula OM-Oxy-Mask
PRM-Partial Rebreather Mask RA-Room Air SM-Simple Mask T-T-Piece VM-Venti Mask VNT-Ventilator O-Other

INTAKE/OUTPUT	23-07	07-15	15-23	24 HRS.	23-07	07-15	15-23	24 HRS.	23-07	07-15	15-23	24 HRS.
P.O. ORAL		480	540	1020	240	240		480				
IV FLUIDS		350	1150	1500	1280			1280				
SHIFT TOTAL		830	1690	2520	1520	240		1760				
Voided Urine		600	800	1400	2000	450		2450				
SHIFT TOTAL		600	800	1400	2000	450		2450				